

CME

ACG Clinical Guideline: Colonic Diverticulitis

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Colonic diverticulitis is a common and costly disease, with a lifetime risk of 3%–5%, that drives substantial healthcare use. This guideline provides practical, evidence-based recommendations for the management of diverticulitis in the outpatient gastroenterology setting. Consultations should address diagnosis, typical clinical course, risk factors, the role of antibiotics, and prevention. Computed tomography imaging is essential to confirm the diagnosis of diverticulitis, particularly at the first presentation and in severe cases. After recovery, colonoscopy is recommended for patients with complicated diverticulitis and suggested for those with uncomplicated disease who have alarm symptoms or are not up to date with colorectal cancer screening to rule out an underlying malignancy. Patients treated nonoperatively remain at substantial risk for recurrence, which increases with each episode, whereas complicated diverticulitis most often occurs at the initial presentation. For patients with a history of diverticulitis, lifestyle measures—including a healthy diet (without avoiding nuts, seeds, corn, or popcorn), regular physical activity, achieving or maintaining a healthy weight, avoidance of smoking and heavy alcohol use, and limiting nonsteroidal anti-inflammatory drugs use when possible—may help reduce the recurrence risk. Because randomized trials show little or no benefit, we suggest against routine antibiotics for low-risk patients with acute uncomplicated diverticulitis. Antibiotic use is appropriate for patients with high-risk features, immunocompromise, frailty, or inability to ensure safe outpatient management. Patients with recurrent, uncomplicated diverticulitis that significantly affects quality of life should be referred for surgical consultation to discuss the risks and benefits of elective colon resection.

KEYWORDS: diverticulosis; diverticular disease; diet; obesity; surgery; treatment; diagnosis; prevention

ABBREVIATIONS: ACG, American College of Gastroenterology; CI, confidence interval; CT, computed tomography; HR, hazard ratio; NSAID, nonsteroidal anti-inflammatory drug; OR, odds ratio; RR, risk ratio; SCAD, segmental colitis associated with diverticulosis; SUDD, symptomatic uncomplicated diverticular disease

SUPPLEMENTARY MATERIAL accompanies this paper at <http://links.lww.com/AJG/D934>

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INTRODUCTION

Colonic diverticulitis is a common disease defined by inflammation in and around a diverticulum (1). The lifetime risk of diverticulitis is ~3% in men and 5% in women (2). In the United States, diverticulitis accounts for an estimated 1.7 million clinic visits, 452,000 emergency department visits, and 179,000 hospital admissions each year (3). An estimated 33,000 elective operations for diverticulitis and a similar number of nonelective operations are performed annually (4). The disease is infrequently fatal with 5,754 deaths each year (3). Healthcare expenditures related to diverticulitis are estimated at \$5.5 billion dollars annually in the United States (1,5).

Diverticulitis is a heterogenous disease with several presentations and variable complications (Table 1). The most

common presentation is acute uncomplicated diverticulitis, defined as inflammatory changes in and around a diverticulum. In a population-based study of diverticulitis, 88% of patients presented with acute uncomplicated diverticulitis (1). Complicated diverticulitis is less common and occurs when inflammation is accompanied by a phlegmon, abscess, perforation, peritonitis, stricture, or fistula. An abscess is the most common complication. Although most episodes of diverticulitis resolve within a few weeks, some patients develop chronic or smoldering diverticulitis, defined as inflammation that persists for weeks to months. Recurrent episodes are common in patients with diverticulitis. After an initial episode of diverticulitis, at least one-quarter of patients experience recurrences (1,6). After a second episode, at least half will go on to have a third episode. Patients

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with 3 or more episodes have a high risk of further recurrences. This relapsing and unpredictable course can significantly impair quality of life and contribute to substantial symptom burden.

After an episode of diverticulitis, patients often seek consultation in gastroenterology clinic for management advice. In developing this American College of Gastroenterology guideline, we aimed, where data permit, to provide practical, evidence-based recommendations for diverticulitis patients seen in gastroenterology clinic that reflect advances in the field. Because daily practice decisions often lack such evidence, we included key concepts to offer practical guidance and expert opinion. These guidelines are intended to support clinical practice and suggest preferred approaches based on current literature. Clinicians should integrate patient comorbidities, overall health, and preferences to ensure patient-centered care.

METHODS

Under the direction of the American College of Gastroenterology (ACG) Practice Parameters Committee, experts in colonic diverticulitis and Grading of Recommendations, Assessment, Development, and Evaluation (GRADE) methodology were selected to form the writing group. The proposed group was reviewed by the ACG Practice Parameters Committee and the ACG Board of Trustees, and the final approved writing group consisted of the current authors.

Regular meetings were held throughout the guideline development process to formulate population, intervention, comparator, outcome (PICO) questions that guided the literature search, the GRADE assessments, the development of recommendation statements, and key concepts, and the preparation of the full guideline document. The PICO questions are included in Supplementary Table 1 (see Supplementary Digital Content, <http://links.lww.com/AJG/D934>). Electronic literature searches were conducted in PubMed (MEDLINE) published from January 1 2000 through January 30, 2025. Searches were limited to English-language, studies involving human participants with a focus on the highest levels of evidence. Priority was given to systematic reviews and meta-analyses, followed by randomized controlled trials when available.

These guidelines are presented as statements identified by the content authors as clinically important. The GRADE process was used to assess the certainty of evidence for each statement (Table 2) (7–10). Evidence certainty is rated as high (we are confident in the effect estimate), moderate, low, or very low (we have very little confidence in the effect estimate), based on factors such as risk of bias, publication bias, heterogeneity, directness of the evidence, and precision of the effect estimate (8).

Each statement is also assigned a strength of recommendation—either strong (recommend) or conditional (suggest) based on the certainty of evidence, the balance of benefits and risks, feasibility, and costs, taking into account both patient-level and

Table 1. Definitions of colonic diverticulosis and diverticulitis

Condition	Definition
Diverticulosis	Diverticulosis refers to outpouchings of the colon that form when the mucosa and submucosa herniate through the muscularis propria, typically at weak points where the vasa recta penetrate the wall.
Acute uncomplicated diverticulitis	Acute uncomplicated diverticulitis is defined as localized inflammation of colonic diverticula without associated abscess, perforation, stricture, or fistula. CT imaging typically shows colonic wall thickening and/or increased pericolic fat stranding.
Smoldering or chronic diverticulitis	Smoldering or chronic diverticulitis is defined as inflammation of colonic diverticula that persists for weeks to months. These patients often symptomatically improve partially or completely, only to relapse shortly after antibiotics are stopped. An abscess is sometimes associated with this manifestation of diverticulitis.
Complicated diverticulitis	Complicated acute diverticulitis is defined as diverticular inflammation accompanied by phlegmon, abscess, or perforation. The modified hinchey classification is a CT-based system used to grade the severity of acute diverticulitis, especially to guide management decisions. Stage 0—Mild clinical diverticulitis (colonic wall thickening, pericolic inflammation). Stage Ia—Confined pericolic inflammation or phlegmon. Stage Ib—Pericolic or mesocolic abscess. Stage II—Pelvic, intra-abdominal, or retroperitoneal abscess. Stage III—Generalized purulent peritonitis. Stage IV—Generalized fecal peritonitis. Stages 0–I are typically managed nonoperatively, whereas higher stages often require drainage or surgery, depending on severity and patient factors Strictures and fistulas may develop as delayed complications.
SCAD	SCAD is a chronic inflammatory condition affecting colonic segments with diverticula, typically sparing the rectum. The inflammation is usually centered on and around the diverticula, and the histologic appearance can resemble ulcerative colitis or Crohn's disease. Whether SCAD represents a distinct entity or a manifestation of inflammatory bowel disease remains unclear (79).
SUDD	SUDD is a controversial diagnosis, sometimes defined as bloating, chronic/recurrent abdominal pain, or altered bowel habits (diarrhea or constipation) in individuals with diverticulosis, in the absence of systemic inflammation or overt diverticulitis (80,81).

CT, computed tomography; SCAD, Segmental colitis associated with diverticulosis; SUDD, Symptomatic uncomplicated diverticular disease.

population-level considerations (11). In addition, a narrative evidence summary for each section provides further details supporting the statements.

In addition to guideline recommendations (Table 3), we developed key concept statements (Table 4) that were not evaluated using the GRADE process. Key concepts include statements based on expert opinion as well as definitions and epidemiologic information to which GRADE methodology does not apply. To help patients understand their condition, make informed decisions, and actively participate in their care, we have included a patient summary written at an 8th grade reading level (Table 5).

These guidelines are intended to support clinical practice by suggesting preferred approaches to the typical patient with a given medical problem, based on the best available published evidence. When exercising clinical judgment—especially when treatments carry significant risks—clinicians should consider this guideline alongside the patient's medical comorbidities, overall health status, and preferences to ensure a patient-centered approach to care.

Recommendations and summary of evidence

Recommendation statements are detailed below and summarized in Table 3. Key concepts in diagnosis and clinical course are described in Table 4 and explained in the Discussion section of the guideline. A summary for patients is included in Table 5.

- 1. We recommend colonoscopy after recovery from an episode of complicated diverticulitis to rule out a missed cancer or premalignant lesion** (*strong recommendation, moderate certainty evidence*).
- 2. We suggest colonoscopy after recovery from an episode of uncomplicated diverticulitis to rule out a missed cancer or premalignant lesion in patients with alarm symptoms or in those not current with colon cancer screening** (*conditional recommendation, low certainty evidence*).

Summary of evidence

Colon cancer can be misdiagnosed as diverticulitis, although the absolute risk is low. Patients with an initial diagnosis of complicated diverticulitis are at the greatest risk. In a meta-analysis of 19 observational studies, the overall rate of colon cancer after an initial diagnosis of diverticulitis—complicated or uncomplicated—was 1.6% (95% confidence intervals [CIs] 1.1%–2.1%) (12). The rate of advanced colonic neoplasia ranges from 5.5% to 6.7%, although results were not stratified by disease severity. Compared with uncomplicated diverticulitis, complicated diverticulitis was associated with a 6-fold higher odds of colon cancer (odds ratio [OR] 5.65, 95% CI 2.95–10.8) and fourfold higher odds of advanced neoplasia (OR 3.85, 95% CI 2.11–7.03). Cancers were generally located in the same region as the presumed diverticulitis, supporting misdiagnosis. In a national US colonoscopy registry, outpatients with an indication of complicated diverticulitis had 4 times the risk of colon cancer compared with a screening population (OR 3.57, 95% CI 1.59–8.01) (13). By contrast, patients with uncomplicated diverticulitis had cancer risks similar to the screening population (OR 1.57, 95% CI 0.70–3.51) and were actually less likely to have advanced neoplasia (OR 0.68, 95% CI 0.52–0.90).

Because patients with recently treated complicated diverticulitis are at an increased risk of missed cancer or advanced neoplasia, follow-up colonoscopy is recommended. Complicated diverticulitis includes phlegmon or abscess, any perforation, stricture, or fistula. After uncomplicated diverticulitis, colonoscopy should be considered for patients who are not current with colorectal cancer screening or who have alarm features such as unintentional weight loss, changes in bowel habits, iron-deficiency anemia, bloody stools, or persistent abdominal pain.

Colonoscopy should not be performed within 6–8 weeks of an acute diverticulitis episode, as inflammation, edema, and luminal narrowing can make a complete examination difficult and increase the risk of complications. The risk of incomplete colonoscopy is 3.7% (95% CI 2.7%–4.9%) and the risk of perforation is 0.2% (12).

3. We suggest against routine antibiotics for low-risk patients with acute uncomplicated diverticulitis. Antibiotic use is appropriate for patients with high-risk features, immunocompromise, frailty, or inability to ensure safe outpatient management (*conditional recommendation, moderate certainty evidence*).

Summary of evidence

For decades, antibiotics were the standard of care for all patients with acute diverticulitis. Recent evidence has challenged the assumption that all patients with acute uncomplicated disease require antibiotics. Four randomized trials including 1,809 patients evaluated the effectiveness and safety of antibiotics for mild, left-sided acute uncomplicated diverticulitis (14–17). In a meta-analysis of these trials, antibiotics had little or no effect on reducing mortality (risk ratio [RR] 1.03, 95% CI 0.22–4.82), progression to complicated diverticulitis (RR 0.72, 95% CI 0.35–1.50), or need for surgery (RR 0.71, 95% CI 0.35–1.44) (18). Antibiotics also had little or no impact on readmission (RR 0.86, 95% CI 0.47–1.58) or recurrent diverticulitis (RR 0.95, 95% CI 0.67–1.34).

Based on these findings, we suggest selective rather than routine antibiotic use in patients with acute uncomplicated diverticulitis (Table 6). This approach is consistent with guidance from the American College of Physicians, which outlines explicit criteria for identifying patients unlikely to benefit from antibiotics (19). These low risk patients have no evidence of complications, have not recently received antibiotics, are immunocompetent, lack signs of systemic inflammatory response, are not medically frail, do not require hospitalization, and are able to reliably follow up as outpatients.

Importantly, after a diagnosis of acute uncomplicated diverticulitis, ~5% of patients develop a complication within 3 months, with most occurring in the first 10 days. High-risk features associated with subsequent complications include significant comorbidities, symptom duration longer than 5 days, vomiting, elevated C-reactive protein levels (>140 mg/L), fluid collection on imaging, and a longer segment of colonic inflammation (20,21). These clinical and radiographic factors should be considered when individualizing decisions regarding antibiotic use. When antibiotics are indicated for outpatient treatment of acute uncomplicated diverticulitis, several

Table 2. Grading of recommendations, assessment, development, and evaluation: Strength of recommendations, certainty of evidence, and stakeholder implications

Strength of recommendation	Criteria
Factors influencing the strength of the recommendation include the certainty of the evidence, clinical and patient-reported outcomes, risk of harm, and costs/healthcare resource utilization.	
Strong	Strong recommendations are offered when the desirable effects of an intervention clearly outweigh the undesirable effects. Implications from a patient and clinician perspective. Patients: Most individuals in this situation would prefer the recommended course of action and only a small proportion would choose an alternative. Clinicians: Most patients should receive the recommended course of action or an alternative with similar strength of recommendation.
Conditional	Conditional recommendations are offered when trade-offs are less certain—either because of low certainty evidence or because evidence suggests that desirable and undesirable effects are closely balanced. Implications from a patient and clinician perspective. Patients: Some individuals would want the suggested course of action whereas others may not. A discussion regarding pros, cons, and available alternatives is appropriate to reach an individualized patient-specific decision. Clinicians: A shared decision-making model through a discussion regarding the available evidence and alternative options is appropriate, taking into consideration the values and preferences of the patient.
Certainty of evidence	Criteria
High	We are very confident that the true effect closely aligns with that of the estimate of the effect.
Moderate	We have a moderate level of confidence in the estimate of effect. It is likely that the true effect is close to the estimate of the effect.
Low	Our confidence in the effect estimate is limited. The true effect could differ from the estimate of effect.
Very low	We have very little confidence in the effect estimate. The true effect may be substantially different from the estimate of effect.

appropriate regimens are available (Table 6) and are discussed in the Discussion section.

4. We suggest a healthy diet (high in fruits, vegetables, whole grains and legumes with limited consumption of red meat, processed grains, trans fats, and sweets) in patients with a history of diverticulitis to reduce the risk of recurrence
(conditional recommendation, very low certainty evidence).

Summary of evidence

Dietary intake influences the risk of diverticulitis. Studies have examined both individual dietary components—such as fiber and red meat— and broader dietary patterns that reflect long-term eating habits before diagnosis.

In a large prospective cohort of US men, those in the highest quintiles of a prudent diet pattern and Alternative Healthy Eating Index had lower risks of incident diverticulitis compared with those in the lowest quintiles (hazard ratio [HR] 0.74, 95% CI 0.60–0.91 and RR 0.67, 95% CI 0.55–0.82, respectively) (22). Similarly, in a cohort of women, higher adherence to the Dietary Approaches to Stop Hypertension diet (HR 0.77, 95% CI 0.65–0.90), Health Eating Index (HR 0.78, 95% CI 0.66–0.91), Alternative Health Eating Index (HR 0.81, 95% CI 0.69–0.95) and Alternative Mediterranean diet (HR 0.91, 95% CI 0.78–1.06) had reduced risk of incident diverticulitis (23). These dietary patterns

emphasize fruits, vegetables and whole grains and limit red meat, refined grains, unhealthy fats, and sweets.

In the EPIC-Oxford study, vegetarians had a 31% lower risk of diverticular disease than meat eaters (HR 0.69, 95% CI 0.55–0.86) (24). High red meat intake increased risk (HR 1.58, 95% CI 1.19–2.11 for highest vs lowest quintile), whereas poultry and fish did not (25).

Higher dietary fiber intake, particularly from cereals and fruits, is consistently associated with lower risk (26,27). In the EPIC-Oxford study, those in the highest quintile of fiber intake had a 41% lower risk than those in the lowest quintile (HR 0.59, 95% CI 0.46–0.78) (24). In a US cohort of women, higher intakes of total fiber (HR 0.86, 95% CI 0.78–0.95), cereal fiber (HR 0.89, 95% CI 0.81–0.99), and fruit fiber (HR 0.83, 95% CI 0.75–0.92), but not vegetable fiber (HR 0.91, 95% CI 0.83–1.01), were associated with a lower risk (27).

Most studies have evaluated diet and incident diverticulitis rather than recurrence. Three studies have examined dietary modifications for prevention of recurrence, but their poor methodical quality limits conclusions (28,29). Nonetheless, the protective effects of a healthy diet likely extend to individuals with previous diverticulitis and have other health benefits as well.

Based on available evidence, patients recovered from diverticulitis should be advised to consume a high-fiber diet rich in fruits, vegetables, whole grains, and legumes and to limit red meat, processed grains, trans fats, and sweets to reduce the risk of recurrence. Supplemental fiber is not a substitute for a high-quality diet. The mechanism by which diet reduces diverticulitis

Table 3. Summary of diverticulitis recommendations

Statement	Certainty of evidence	Strength of recommendation
1. We recommend colonoscopy after recovery from an episode of complicated diverticulitis to rule out a missed cancer or premalignant lesion.	Moderate	Strong
2. We suggest colonoscopy after recovery from an episode of uncomplicated diverticulitis to rule out a missed cancer or premalignant lesion in patients with alarm symptoms or in those not current with colon cancer screening.	Low	Conditional
3. We suggest against routine antibiotics for low-risk patients with acute uncomplicated diverticulitis. Antibiotic use is appropriate for patients with high-risk features, immunocompromise, frailty, or inability to ensure safe outpatient management.	Moderate	Conditional
4. We suggest a healthy diet (high in fruits, vegetables, whole grains and legumes with limited consumption of red meat, processed grains, trans fats and sweets) in patients with a history of diverticulitis to reduce the risk of recurrence.	Very low	Conditional
5. In patients with a history of diverticulitis, we suggest against the avoidance of nut, corn, seed or popcorn consumption to reduce the risk of recurrence.	Very low	Conditional
6. Unless there is a clinical indication, we suggest patients avoid regular use of nonsteroidal anti-inflammatory drugs (NSAIDs) after recovery from an episode of diverticulitis to reduce the risk of recurrence.	Low	Conditional
7. In patients with a history of diverticulitis who smoke, we suggest smoking cessation to reduce the risk of recurrence.	Very low	Conditional
8. In patients with a history of diverticulitis who consume heavy alcohol, we suggest moderation to reduce the risk of recurrence.	Very low	Conditional
9. In patients with a history of diverticulitis who are overweight or obese, we suggest weight loss to reduce the risk of recurrence.	Low	Conditional
10. We suggest regular physical activity in patients with a history of diverticulitis to reduce the risk of recurrence.	Very low	Conditional
11. In patients with recurrent, uncomplicated diverticulitis that significantly impairs quality of life, we suggest surgical referral to discuss the risks and benefits of elective colon resection.	Low	Conditional
12. In patients with a history of acute diverticulitis, we recommend against the use of probiotics to reduce the risk of recurrence.	Very low	Conditional
13. We recommend against the use of mesalamine in patients with a history of acute diverticulitis to reduce the risk of recurrence.	Moderate	Strong
14. We recommend against the use of rifaximin in patients with a history of acute diverticulitis to reduce the risk of recurrence.	Very low	Conditional

risk remains unclear; constipation is no longer presumed to play a role (30).

5. In patients with a history of diverticulitis, we suggest against the avoidance of nut, corn, seed, or popcorn consumption to reduce the risk of recurrence (*conditional recommendation, very low certainty evidence*).

Summary of evidence

Patients with diverticulosis or diverticulitis often avoid nuts, seeds, corn, and popcorn, although evidence does not support this practice. In a cohort of 47,228 men, the hazards ratios for incident diverticulitis when comparing highest to lowest intake categories were 0.80 (95% CI 0.63–1.01) for nuts, 0.72 (95% CI 0.52–0.92) for popcorn, and 1.13 (95% CI 0.83–1.54) for corn (31). Similarly, in a prospective cohort of

29,916 women, higher intake of peanuts, nuts and seeds (adjusted hazard ratio [aHR] 1.07, 95% CI 0.91–1.25) or of fruits with small seeds such as tomatoes and strawberries (aHR 1.06, 95% CI 0.90–1.24) was not associated with an increased risk (23). Findings in both studies were robust to sensitivity analyses. Although data on nut and seed consumption and recurrent diverticulitis are lacking, the effects are likely similar to those for incident disease. Based on available evidence, patients with a history of diverticulitis should not avoid consuming nuts, corn, seeds, or popcorn to prevent recurrence.

6. Unless there is a clinical indication, we suggest patients avoid regular use of nonsteroidal anti-inflammatory drugs (NSAIDs) after recovery from an episode of diverticulitis to reduce the risk of recurrence (*conditional recommendation, low certainty evidence*).

Table 4. Key concepts in diverticulitis diagnosis and clinical course

Diagnosis	Because clinical evaluation alone is inaccurate in most cases, CT is useful—especially at first presentation—to confirm suspected diverticulitis, exclude alternative diagnoses, grade disease severity to guide treatment, and identify the affected location when surgical management is being considered (54).
Risk of diverticulitis	Among individuals with diverticulosis, the risk of developing diverticulitis is low (~1%–4% over 7 yr of follow up) (67).
Risk of complicated diverticulitis	The risk of complicated diverticulitis is highest with the initial presentation and decreases significantly with subsequent recurrences (1).
Risk of recurrence	Patients with a history of diverticulitis managed nonoperatively remain at high risk for recurrence: after a first episode the 10-yr risk of a second episode is at least 22%; after a second episode the 10-yr risk of a third episode rises to at least 55%; and after 3 or more episodes the likelihood of further recurrence remains high (1).
Risk of diverticulitis after elective surgery	Elective surgery reduces but does not eliminate the risk of recurrence: the 5-yr recurrence rate is ~60% without surgery vs approximately 15% after elective colectomy (42).
Risk of chronic gastrointestinal symptoms	Chronic gastrointestinal symptoms, including abdominal pain and altered bowel habits, are common after recovery from diverticulitis, with more than one-third of patients reporting persistent symptoms 1–2 yr after an episode (77).
CT, computed tomography.	

Summary of evidence

Nonsteroidal anti-inflammatory drugs (NSAIDs), including aspirin, are widely used and readily available drugs with a number of important indications. There is evidence, albeit limited, that NSAID use is associated with an increased risk of incident diverticulitis (32). In a large prospective cohort study of approximately 47,000 men (33), regular use of nonaspirin NSAIDs was associated with an increased risk of diverticulitis (HR 1.72, 95% 1.40–2.11). Aspirin use was also associated with an increased risk although the association was weaker (HR 1.25, 95% 1.05–1.47). No studies have evaluated NSAIDs and the risk of recurrence, but the effect is presumed to be similar to incident disease. Patients should be informed that very limited evidence suggests regular NSAID use is associated with an increased risk of diverticulitis. Patients should not avoid NSAIDs use if there is a compelling indication for use.

7. In patients with a history of diverticulitis who smoke, we suggest smoking cessation to reduce the risk of recurrence (*conditional recommendation, very low certainty evidence*).

Summary of evidence

Cigarette smoking increases the risk of diverticulitis, with a stronger association for complicated disease. In a meta-analysis of 6 prospective cohort studies, current smokers had a 36% higher risk of diverticular disease resulting in hospitalization or death compared with never smokers (RR 1.36, 95% CI 1.15–1.61). In 2 studies evaluating complicated diverticulitis (abscess or perforation), current smokers had a 2.5-fold higher risk compared with never smokers (RR 2.54, 95% CI 1.49–4.33) (34). A separate prospective study of women found similar results: current smokers had an increased risk of incident diverticulitis (HR 1.20, 95% CI 1.04–1.39), with a stronger association among those requiring surgery (HR 1.48, 95% CI 1.04–2.12), although without a dose-response relationship (35). No studies have evaluated

smoking and the risk of recurrence, but the effect is presumed to be similar to incident disease. Given the available evidence and the well-established harms of smoking, we suggest smoking cessation for patients with a history of diverticulitis who smoke to reduce the risk of recurrence.

8. In patients with a history of diverticulitis who consume heavy alcohol, we suggest moderation to reduce the risk of recurrence (*conditional recommendation, very low certainty evidence*).

Summary of evidence

Evidence suggests that heavy alcohol use increases the risk of diverticulitis. In a large prospective cohort of US women, those with the highest alcohol intake (>30 g per day; >2 drinks per day) had a higher risk of incident diverticulitis compared with nondrinkers (HR 1.26, 95% CI 1.05–1.50) (35). Liquor and white wine were positively associated with risk, whereas red wine and beer were not. Women who both smoked and consumed at least 15 g/d of alcohol had a 60% higher risk of diverticulitis. Alcohol was not associated with diverticulitis requiring surgery.

In the EPIC-Oxford cohort of men and women, alcohol intake >16 g/d was associated with higher diverticulitis risk compared with nondrinking, but this association was no longer significant after adjusting for smoking (24).

These studies evaluated incident—not recurrent—diverticulitis, but the effect of alcohol is presumed to be similar. Given evidence that heavy alcohol consumption increases risk, we suggest that patients with a history of diverticulitis consume alcohol in moderation.

9. In patients with a history of diverticulitis who are overweight or obese, we suggest weight loss to reduce the risk of recurrence (*conditional recommendation, low certainty evidence*).

Table 5. Education for patients with diverticulosis and diverticulitis

Colonic diverticulitis—what you should know	
What causes diverticulitis?	Diverticulitis is a common condition caused by inflammation of small pouches in the colon called diverticula. These pouches form over time as weak spots develop in the wall of the colon. Most older adults have diverticulosis but <1 in 10 will ever develop diverticulitis. Several factors can increase the risk of developing diverticulitis. These include being overweight, smoking, heavy alcohol use, not getting enough exercise, and long-term eating habits such as diets high in red meat, processed foods, unhealthy fats, and sweets. Some people are more likely to develop diverticulitis because of their genes. Even so, living a healthy lifestyle—eating a balanced diet, staying active, and avoiding smoking—can help lower the risk of diverticulitis for most people.
What is uncomplicated diverticulitis? What is complicated diverticulitis?	Acute uncomplicated diverticulitis is a sudden flare of inflammation in small pouches in the colon. It means the inflammation is limited to the colon and has not caused a pocket of infection, a hole in the bowel, a narrowing, or an abnormal connection to another organ. Complicated diverticulitis is a more serious form of diverticulitis that has caused a problem such as a pocket of infection, a hole in the bowel, a blockage or narrowing of the colon, or an abnormal connection to another organ. The risk of complications is highest during the first episode of diverticulitis. Most people with diverticulitis have uncomplicated diverticulitis, and serious problems are uncommon.
Is diverticulitis caused by constipation? Do I need to take a laxative?	Constipation is no longer thought to cause diverticulosis or diverticulitis. There is no need to take a laxative unless you have chronic constipation or your doctor recommends it.
Should I avoid nuts, seeds, corn, and popcorn?	You do not need to avoid nuts, seeds, corn, or popcorn if you have diverticulosis or have had diverticulitis. Large studies show that eating these foods does not increase the risk of diverticulitis or make it come back. In fact, people who ate nuts or popcorn were not more likely to have problems than those who did not. Avoiding these foods does not prevent diverticulitis, so you can include them as part of a healthy diet unless they bother you personally.
Will I have another attack?	If you have had diverticulitis and did not need surgery, there is a chance it can come back. Approximately 2 out of 10 people will have another episode within 10 yr after the first 1. After a second episode, the chance of having a third episode goes up to approximately 5 out of 10. Once someone has had several episodes, the risk of future attacks stays high.
I've had a few episodes of diverticulitis. Will I someday develop a complication and need a colostomy bag?	A colostomy bag is a pouch worn on the belly that collects stool after surgery creates a new opening for it to leave the body. The risk of complications is highest during the first episode of diverticulitis. Most people with diverticulitis have a mild form called uncomplicated diverticulitis, and serious problems are uncommon. If diverticulitis comes back later, it is less likely to cause serious problems, and needing a colostomy bag is very unlikely.
What should I eat during an episode of diverticulitis?	If you have mild, uncomplicated diverticulitis, some people feel better by drinking only clear liquids for a few days. There is no proof that this helps the diverticulitis heal faster, but it may help ease discomfort at first. This may be because swelling in the colon can make it harder and more painful to have a bowel movement and can cause more belly pain.
What should I eat when I'm recovered from the episode?	Eating healthy foods may help lower your chance of getting diverticulitis again. Foods like fruits, vegetables, whole grains, and beans may help protect against future attacks. It may also help to eat less red meat, processed foods, sweets, and foods high in unhealthy fat. If you start eating a lot of fiber too quickly after a diverticulitis episode, it can sometimes cause cramping or belly discomfort. It is better to add fiber slowly over time. Fiber pills or powders are not as helpful as getting fiber from food, so try to focus on high-fiber foods when you can.
What else can I do to prevent diverticulitis?	Staying physically active, keeping a healthy weight, not smoking, and limiting alcohol may help lower the chance that diverticulitis comes back.
Is it OK to take NSAIDs?	After you recover from diverticulitis, it is best to avoid taking pain medicines called NSAIDs on a regular basis unless your doctor tells you to use them. NSAIDs include medicines like ibuprofen, naproxen, and aspirin. Some studies suggest that these medicines may raise the chance of getting diverticulitis again. Acetaminophen (tylenol) is not an NSAID and is usually okay to take for pain or fever. If you take NSAIDs for another health reason, such as arthritis, do not stop them on your own—talk with your doctor about what is safest for you.

Table 5. (continued)

Colonic diverticulitis—what you should know	
Should I take a probiotic?	Probiotics are good bacteria found in some foods and supplements. Some people take them to try to prevent diverticulitis from coming back. However, studies have not shown that probiotics help prevent another episode of diverticulitis. In 1 study, people who took probiotics did not do better than those who did not. Because probiotics do not seem to help and can be costly or inconvenient to take every day, we do not recommend using them to prevent diverticulitis from coming back.
Do I need a colonoscopy?	After an episode of diverticulitis, your doctor may recommend a colonoscopy to make sure there is no hidden colon cancer or growth. This is especially important if your diverticulitis was complicated (such as with an abscess, hole, or blockage), because the chance of cancer is higher in these cases. If your diverticulitis was uncomplicated, you may still need a colonoscopy if you are not up to date on colon cancer screening or if you have warning signs such as weight loss, blood in the stool, anemia (which means a low red blood cell count), changes in bowel habits, or ongoing belly pain. A colonoscopy should usually be performed 6–8 wk after recovery, not during the acute attack, to lower the risk of problems and allow a safe, complete examination.
Should I take antibiotics when I have a diverticulitis episode?	If you have mild, uncomplicated diverticulitis, antibiotics are usually not needed except in certain circumstances. Studies show that for many healthy people with mild symptoms, antibiotics do not speed recovery or prevent problems. Instead, symptoms often improve with time and close follow-up with your doctor. Antibiotics may still be used if symptoms are severe, if you have other health problems, or if complications develop. Your doctor will help decide what is safest and best for you.
Should I have surgery?	If you have repeated episodes of uncomplicated diverticulitis that significantly affect your daily life, your doctor may recommend meeting with a surgeon to discuss whether surgery is right for you. Surgery can reduce the chance of future attacks and may improve quality of life, but it does not guarantee that diverticulitis will not come back. It also carries risks, including infection, bowel complications, and, in some cases, the need for an ostomy—a surgically created opening in the abdomen that allows stool to leave the body into a bag. Because of these risks, the decision should be made carefully and should take into account your symptoms, test results, overall health, and personal goals. Surgery should not be recommended simply because you are young or because you have had a certain number of attacks, and it is not advised solely to prevent a future diverticulitis emergency.
NSAID, nonsteroidal anti-inflammatory drugs.	

Summary of evidence

Obesity is a risk factor of diverticulitis, as demonstrated in multiple observational studies (36–39). Increasing severity of obesity is associated with a greater risk (36,38). Obesity also seems to increase the risk of recurrent diverticulitis (37). Women with a body mass index ≥ 30.0 kg/m² had an increased risk of recurrent diverticulitis compared with women with a body mass index < 22.5 kg/m² (HR 1.66, 95% CI, 1.09–2.51). Emerging evidence suggests that visceral adiposity—fat stored around internal organs in the abdominal cavity—may be a more important risk factor of diverticulitis compared with overall excess body fat (39,40). However, it remains unknown whether weight loss reduces recurrence risk. Although the evidence is limited, it is reasonable to advise patients who are overweight or obese and recovered from diverticulitis to pursue weight loss with the goal of achieving a healthy body mass index.

10. We suggest regular physical activity in patients with a history of diverticulitis to reduce the risk of recurrence (*conditional recommendation, very low certainty evidence*).

Summary of evidence

Regular physical activity is also associated with a reduced risk of incident diverticulitis. In a meta-analysis of 5 observational studies, high physical activity was linked to a lower risk of diverticulitis compared with low activity (RR 0.76, 95% CI 0.63–0.93) after adjustment for confounding variables (36). These studies evaluated physical activity and incident diverticulitis rather than recurrence, but the effects are likely similar. Based on available evidence, patients with a history of diverticulitis should engage in regular physical activity to help prevent recurrence.

11. In patients with recurrent, uncomplicated diverticulitis that significantly impairs quality of life, we suggest surgical referral to discuss the risks and benefits of elective colon resection (*conditional recommendation, low certainty evidence*).

Summary of evidence

The decision to undergo elective surgery for recurrent diverticulitis should be individualized. Clinicians should refer patients with recurrent, uncomplicated diverticulitis that significantly impairs

Table 6. Approach to antibiotic use in patients with acute uncomplicated diverticulitis

Patients with acute uncomplicated diverticulitis appropriate for initial nonantibiotic management are those who	All of the following must be present • Immunocompetent • Outpatient • Have no evidence of complicated disease • Hemodynamically stable • No signs of systemic inflammatory response (fever, tachycardia, leukocytosis) • Not medically frail • Able to tolerate oral intake • No recent antibiotic use • Reliable follow-up and adequate social support
Antibiotics are reasonable for patients with acute uncomplicated diverticulitis who have	• Immunocompromise • Frailty • Significant comorbid conditions • Clinical deterioration or persistent/worsening symptoms • Inability to tolerate oral intake • Vomiting • Elevated inflammatory markers (e.g., markedly elevated C-reactive protein) • Imaging features associated with higher risk (e.g., fluid collection or longer segment of inflammation) • Inadequate outpatient support or inability to ensure close follow-up
When antibiotics are used for outpatient treatment of acute uncomplicated diverticulitis, several options are appropriate	Monotherapy with • Amoxicillin-clavulanic acid 875 mg/125 mg orally every 12 h • Moxifloxacin 400 mg orally daily Combination therapy with • Metronidazole 500 mg orally every 8 h plus • Trimethoprim-sulfamethoxazole (1 double strength tablet, 160/800 mg) every 12 h or • Ciprofloxacin 500 mg every 12 h or • Levofloxacin 500 mg every 24 h

quality of life for surgical consultation. The potential benefits of surgery, such as a reduced risk of future diverticulitis and improved quality of life, must be weighed against the potential harms, including surgical complications. Previously, elective colectomy was recommended after 2 episodes of acute uncomplicated diverticulitis, but this approach is no longer supported by evidence (41). Recurrent diverticulitis does not increase the risk of developing complicated diverticulitis and elective surgery is no longer recommended to prevent emergency operations. The number of episodes should not determine the need for surgery.

Elective surgery reduces, but does not eliminate, the risk of recurrence. The 5-year recurrence risk is 15% after elective colectomy compared with 60% without surgery (42). Two randomized trials (DIRECT and LASER) demonstrated improved quality of life with surgery; however, both studies were small and included younger, healthier patients than the general diverticulitis population (43,44). Although there was an improvement in quality of life measures in patients who underwent elective surgery in both trials, it is important to note for patient counseling purposes that quality of life did not improve or was worse in 30% of patients in the surgical treatment group in the DIRECT Trial.

Surgery also carries important risks. Short-term complications include surgical site infections, iatrogenic injury, and anastomotic leak, whereas long-term risks include incisional hernia, permanent ostomy, anastomotic strictures, adhesive small bowel obstruction, and new-onset bowel dysfunction (4,45,46).

Major surgical complications were common (10%–34%) in both the DIRECT and LASER Trials (43,44).

Computed tomography confirmation of diverticulitis is essential before considering elective resection. Patients with chronic gastrointestinal symptoms should have realistic expectations, recognizing that abdominal pain may persist after surgery. In the LASER trial, 73% of patients reported baseline abdominal pain and 63% continued to experience pain 6 months postoperatively (47).

Historically, elective resection was often recommended in young adults after a single episode; however, observational studies now show that younger age does not confer a high risk of recurrence or complicated disease (48). Therefore, young age alone is not an indication for surgery.

This recommendation does not apply to patients with strictures, fistulas, chronic diverticulitis, or a history of diverticulitis complicated by abscess or perforation.

12. In patients with a history of acute diverticulitis, we recommend against the use of probiotics to reduce the risk of recurrence (conditional recommendation: very low certainty evidence).

Summary of evidence

Probiotics are live microorganisms found in fermented foods and dietary supplements. One randomized trial evaluated a probiotic

supplement and found no benefit. The study compared combination mesalamine plus probiotics, mesalamine alone, and placebo in 117 patients with imaging-confirmed diverticulitis; the risk of recurrent diverticulitis was similar across all groups over 1 year of follow up (49).

Several studies have examined probiotics in symptomatic uncomplicated diverticular disease; however, these findings are not applicable to patients with diverticulitis. Overall, there is no evidence that probiotics benefit patients with a history of diverticulitis. The cost, potential harms, and burden of daily probiotics use were additional important considerations in our recommendation.

13. We recommend against the use of mesalamine in patients with a history of acute diverticulitis to reduce the risk of recurrence
(*strong recommendation, moderate certainty evidence*).

Summary of evidence

Clinicians and patients desire a medical treatment that reduces the risk of recurrent diverticulitis. Because diverticulitis is characterized by inflammation, mesalamine (5-aminosalicylic acid)—an anti-inflammatory agent used in ulcerative colitis with a long and reassuring safety record—was an appealing candidate. Early trials, primarily conducted in Europe, suggested potential benefits of mesalamine for patients with a history of diverticulitis. However, the US DIVA trial and the larger PREVENT-1 and PREVENT-2 trials did not demonstrate efficacy (49,50). Subsequent international trials (SAG-37 and Sag-51) also failed to show benefit (51).

The most recent and comprehensive systematic review and meta-analysis, published by the American College of Physicians in 2022, evaluated the use of mesalamine in patients with previous diverticulitis (12). The review included 6 randomized controlled trials comparing various doses of mesalamine with placebo in 1,898 patients (12). Mesalamine did not reduce the risk of recurrence compared with placebo (OR 1.15, 95% CI 0.92–1.44). Discontinuation due to adverse events was more common with mesalamine (OR 1.59, 95% CI 1.12–2.27). There were no differences in symptoms scores. Although not formally addressed in the review, cost and the burden of daily medication use were additional considerations in our recommendation.

14. We recommend against the use of rifaximin in patients with a history of acute diverticulitis to reduce the risk of recurrence
(*conditional recommendation, very low certainty evidence*).

Summary of evidence

Rifaximin is an antibiotic used for several gastrointestinal conditions and 1 small trial tested whether 1 week of rifaximin per month could reduce recurrent diverticulitis (52). In 165 patients, combination rifaximin plus fiber showed lower recurrence at 1 year than fiber alone (10% vs 19% recurrence), but the study was open-label, unclear in outcome definitions, and stopped early due to poor recruitment. A retrospective cohort study also suggested reduced recurrence with rifaximin compared with mesalamine, but it had significant methodological limitations (53). Studies of rifaximin in symptomatic uncomplicated diverticular disease do not apply to patients with previous diverticulitis. Overall,

evidence does not support rifaximin for diverticulitis prevention, and its cost and treatment burden were additional considerations in our recommendation.

DISCUSSION

We developed this American College of Gastroenterology guideline with a focus on the outpatient management of patients with diverticulitis seen in gastroenterology clinic. Consultations should address diagnostic accuracy, the risk of recurrence and complications, risk factors and prevention, the role of antibiotics, indications for surgical consultation, and any ongoing gastrointestinal symptoms. Given the prevalence of misinformation, patients with diverticulitis benefit from targeted education. Accordingly, we have included a patient education handout to support informed, evidence-based care (Table 5).

Clinical evaluation alone is often insufficient to accurately diagnose diverticulitis. Certain features—such as older age, previous episodes, left lower abdominal pain and tenderness, pain with movement, lack of vomiting, and elevated C-reactive protein—increase the likelihood of the diagnosis (54–56). Because clinical evaluation alone is inaccurate in most cases, computed tomography is useful—especially at first presentation—to confirm suspected diverticulitis, exclude alternative diagnoses, grade disease severity to guide treatment, and identify the affected location when surgical management is being considered (54).

Colon cancer can sometimes be mistaken for diverticulitis, although the overall risk is low (12). The risk is highest in patients with complicated diverticulitis, who have several-fold higher rates of colon cancer compared with those with uncomplicated disease. Therefore, follow-up colonoscopy is recommended after recovery from complicated diverticulitis and should be considered after uncomplicated diverticulitis for patients with alarm symptoms or in those who are not current with guideline-recommended colorectal cancer screening (57,58). The American College of Physicians guideline (2022) similarly suggested colonoscopy after an initial episode of complicated diverticulitis in patients who had not had a recent high-quality colonoscopy, and noted that uncomplicated or recurrent episodes do not by themselves justify additional colonoscopic evaluation in patients who are otherwise up to date with screening (59).

It is important to understand and educate patients on the typical clinical course of diverticulitis. Most patients present with acute, uncomplicated disease (1). The risk of complicated diverticulitis is highest with the initial presentation and decreases significantly with subsequent recurrences (1). When complications occur, they most often involve an abscess. Patients with a history of diverticulitis managed nonoperatively remain at high risk of recurrence: after a first episode the 10-year risk of a second episode is at least 22%; after a second episode the 10-year risk of a third episode rises to at least 55%; and after 3 or more episodes the likelihood of further recurrence remains high (1). Recurrent diverticulitis can be frustrating because of unpredictable episodes of pain. Recurrence itself does not increase the risk of developing complicated disease.

Patients often want to understand why they have developed diverticulitis and what they can do to prevent recurrence, with dietary guidance being a high priority. Many ask what foods are appropriate during an acute episode and what to eat after recovery. There is limited evidence to guide dietary recommendations during an acute episode managed in the outpatient setting (60). In clinical practice, patients often report benefit from

a clear liquid diet for the first few days of an episode, possibly because inflammation can temporarily narrow the colon, making stool passage more difficult and painful.

Dietary patterns in the years preceding diagnosis play an important role in the risk of developing diverticulitis. Large studies consistently show that healthy dietary patterns—rich in fruits, vegetables, whole grains, legumes, and fiber, and low in red meat, refined grains, unhealthy fats, and sweets—are associated with a lower risk of diverticulitis (22,23). Although fewer studies have examined diet and recurrence, the benefits of a high-quality, high-fiber diet likely extend to patients with previous diverticulitis. Supplemental fiber should not be viewed as a substitute for a healthy diet, and the protective effect of fiber is no longer thought to be explained by treating or preventing constipation (30). There is no evidence to support avoiding nuts, seeds, corn, or popcorn (23,31).

In addition to diet, engaging in regular physical activity, achieving or maintaining a healthy weight, avoiding smoking and heavy alcohol use, and limiting NSAID use when possible—may help reduce the recurrence risk, although the evidence is limited (32,34–40). Although most studies on lifestyle factors evaluated incident diverticulitis rather than recurrence, the effects are likely similar. A large prospective study demonstrated an inverse linear relationship between the number of low-risk lifestyle factors and the incidence of diverticulitis, with adherence to an overall healthy lifestyle associated with approximately a 50% reduction in risk (61). Many patients have a genetic predisposition to diverticulitis. Maintaining a healthy lifestyle is associated with a reduced risk of diverticulitis regardless of genetic susceptibility (62–64).

Diverticulosis is highly prevalent in Western populations (65,66). However, among individuals with diverticulosis, the risk of developing diverticulitis is low (~1%–4% over 7 years of follow-up) (67). Patients interested in prevention can also be advised that modifiable lifestyle measures—including a high-quality, high-fiber diet; regular physical activity; maintaining a healthy weight; avoiding smoking and heavy alcohol use; and minimizing NSAID use when possible—may reduce the risk of diverticulitis, although the absolute risk remains low.

Elective surgery for recurrent, uncomplicated diverticulitis should be considered on an individual basis for patients whose recurrent episodes significantly impair quality of life. Elective surgery reduces but does not eliminate the risk of recurrence: the 5-year recurrence rate is approximately 60% without surgery versus approximately 15% after elective colectomy (42). Although surgery can lower the risk of future episodes and may improve quality of life, it carries important short and long-term risks (4,43–46). The decision to undergo elective colectomy for recurrent diverticulitis requires an informed patient and clear communication about treatment options, potential benefits, and risks. Many patients experience uncertainty when considering elective surgery, and nearly one-third of patients with diverticulitis report significant decision regret after colectomy (68). Shared decision-making that centers on patient-specific benefits and harms may help minimize decision regret.

Multiple guidelines now recommend that clinicians initially manage select patients with acute uncomplicated diverticulitis without antibiotics (19,69–74). Antibiotics may be safely withheld in select, otherwise healthy and stable patients with uncomplicated diverticulitis who can be reliably managed as outpatients. Because a small proportion of patients develop complications—most within the first 10 days—clinical factors

such as significant comorbidities, immunosuppression, prolonged symptoms (>5 days), vomiting, elevated inflammatory markers, and concerning imaging findings can help identify patients who may benefit from antibiotic treatment (20,21).

When antibiotics are used for outpatient treatment of acute uncomplicated diverticulitis, several options are appropriate (75). Options include monotherapy with amoxicillin-clavulanic acid 875 mg/125 mg orally every 12 hours or moxifloxacin 400 mg orally daily. Alternatively, combination therapy with metronidazole 500 mg orally every 8 hours plus trimethoprim-sulfamethoxazole (1 double strength tablet, 160/800 mg) every 12 hours, ciprofloxacin 500 mg every 12 hours, or levofloxacin 500 mg every 24 hours may be used. The usual duration of treatment is 4–7 days. Amoxicillin-clavulanate seems to be as effective as metronidazole plus a fluoroquinolone for outpatient diverticulitis and may reduce the risk of fluoroquinolone-associated harms (including *Clostridioides difficile* infection) without compromising diverticulitis-specific outcomes (76).

Chronic gastrointestinal symptoms, including abdominal pain and altered bowel habits, are common after recovery from diverticulitis, with more than one-third of patients reporting persistent symptoms 1–2 years after an episode (77). Given the broad differential diagnosis for ongoing symptoms, clinicians should consider cross-sectional imaging and colonoscopy to evaluate for complications and alternative diagnoses. Similar to other inflammatory gastrointestinal conditions, diverticulitis may be followed by the development of functional gastrointestinal disorders, also called disorders of gut-brain interaction. Delayed complications such as strictures and fistulas can occur but are uncommon. Strictures typically present with new-onset constipation or obstructive symptoms, whereas fistulas may have variable and sometimes subtle clinical presentations.

Although most episodes of diverticulitis resolve within a few weeks, some patients develop chronic or smoldering diverticulitis, defined as inflammation that persists for weeks to months (78). This manifestation of diverticulitis is poorly understood but not uncommon. Among patients with a new diagnosis of diverticulitis, 6% had smoldering diverticulitis at 6 month follow-up (15). These patients often experience partial or complete symptom improvement with antibiotics, but symptoms recur after antibiotics are discontinued. This pattern differs from recurrent diverticulitis, in which patients have a clear symptom-free and inflammation-free interval between episodes. In these cases, cross-sectional imaging is needed to confirm the diagnosis and exclude alternative diagnoses. In practice, patients are typically treated with antibiotics, often for a prolonged course, and some ultimately require segmental resection for persistent disease.

Several important gaps remain in the evidence, many of which directly affect patient care and the strength of guideline recommendations. Better clinical, biomarker, genetic, and imaging-based predication tools are needed to predict who will recur, who will develop complications, or who will benefit most from antibiotics or elective surgery. There are limited high-quality trials testing dietary changes, lifestyle intervention, or medications to prevent recurrent diverticulitis. Although dietary modification is often recommended after an episode of diverticulitis, robust comparative data are lacking to determine whether a high-fiber diet, a healthy dietary pattern such as the Mediterranean diet, or another approach is most effective for preventing recurrence. There is currently no evidence to guide what or when patients should eat

during an acute, uncomplicated episode in the outpatient setting. Few trials have compared elective surgery versus conservative management in a real-world population. There is a need for better data on patient-centered outcomes, including long-term quality of life and decision regret. Validated decision aids and patient education tools are needed to help patients considering whether to have an elective surgery for diverticulitis. Addressing these gaps would substantially improve personalized care, shared decision-making, and outcomes for patients with diverticulitis.

CONFLICTS OF INTEREST

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